Adjuvant trastuzumab emtansine (T-DM1)

National Cancer Control Programme (NCCP) 00659 · Draft — independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Your practical treatment plan

T-DM1 by intravenous (IV) infusion every 21 days for 14 postoperative cycles, unless recurrence or unacceptable toxicity requires a change.

STEP 1

Before the first visit

Blood tests, cardiac assessment, medicines review and the consent conversation come before T-DM1. Confirm recovery from surgery and the pathology-based treatment plan.

STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

Trained staff check and access the line, give the prescribed premedicines and infusions, and monitor for reactions. Tell the nurse immediately about pain, burning, rash or breathlessness.

STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit time and transport

The first T-DM1 infusion takes 90 minutes, followed by at least 90 minutes of observation. Later tolerated infusions can take 30 minutes, followed by at least 30 minutes of observation. These are infusion and observation times, not the whole visit.

Sources: NCCP 00659 v2 · official regimen · eviQ · implanted port patient information · eviQ · PICC patient information. Draft — independent clinical, pharmacy and patient/PPI review pending.